

776FM.3 DIAGNOSIS AND MANAGEMENT OF POLYMYALGIA RHEUMATICA (PMR)
Target Audience: Primary and Secondary Care

Diagnostic criteria

- Age >50 years
- Symptom duration >2 weeks
- Abrupt onset
- Bilateral shoulder and/or pelvic girdle pain
- Morning stiffness >45 minutes
- Raised **CRP +/- ESR**

Always check for symptoms of Giant Cell Arteritis (GCA)

Differential diagnosis

- Rheumatoid Arthritis, vasculitis, myositis, other inflammatory arthritis
- Infections, i.e. endocarditis, occult and deep sepsis
- Shoulder pathology, i.e. capsulitis, rotator cuff lesions
- Malignancy, i.e. multiple myeloma, metastatic cancer
- Endocrine causes: hypothyroidism, osteomalacia, hyperparathyroidism
- Parkinson's disease
- Chronic pain syndromes
- Drug related myalgia e.g. from statins, quinidine

Laboratory tests prior to steroids

Bloods: ESR and CRP, FBC, U&Es, CK, LFTs, TSH, bone profile, protein electrophoresis
If articular features: RF, ANA, anti CCP antibodies
In cases with prominent systemic symptoms: CXR
Urinalysis to test for haematuria in suspected systemic vasculitis, connective tissue disease, renal cancer

Steroid treatment

Start prednisolone **15 mg daily** (consider trial of 10 mg in patients with lower body mass index (BMI) or milder symptoms whilst 20 mg may be required in patients with partial response or more severe disease). **Also start gastroprotection and give the patient a steroid card.**

Assess the response in 1 week – expect at least 70% improvement in pain global visual analogue scale (VAS)

Continue for **3 - 4** weeks

Then taper dose

12.5 mg for **3 - 4** weeks

10 mg for **4 - 6** weeks

Then reduce by 1 mg every **4 - 8** weeks

Bone protection

Refer to osteoporosis guideline.

[567FM: Fracture prevention for Adults >50 Years Old](#)

Assess the need for bisphosphonate at steroid cessation. If not high risk, stop within 6 months.

Follow-up (4 – 6 weeks)

Likely PMR if:

Normalisation of inflammatory markers
Patient reported global improvement ~ 70%

Symptoms to monitor

Proximal pain
Morning stiffness
Disability related to the PMR
Symptoms that may suggest alternative diagnosis
Osteoarthritis (OA) risk

Glossary:

CRP: C-reactive protein
ESR: Erythrocyte sedimentation rate
FBC: Full blood count
U&Es: Urea & electrolytes
CK: Creatine kinase
LFT: Liver function test
TSH: Thyroid stimulating hormone test
RF: Rheumatoid factor
ANA: Antinuclear antibody
CCP: Cyclic citrullinated peptide antibodies

Long term review of PMR

Aim for review at Weeks 0, 1–3, 6, Months 3, 6, 9, 12 in first year (with extra visits for relapses or adverse events).

Assess

Symptoms of GCA

Steroid-related adverse events

Bloods: FBC, ESR/CRP, U&Es, glucose

Duration of treatment

Usually 1 – 3 years (some will require small doses of steroids beyond this)

Isolated raised ESR/CRP in the absence of PMR/GCA symptoms needs investigating and should not prompt steroid increase



Managing Relapse

Definition: Recurrence of PMR symptoms or new onset of GCA

If GCA

Oral prednisolone 40 mg – 60 mg daily – see GCA guideline

[194FM Giant Cell Arteritis Guideline](#)

If features of PMR only

Increase prednisolone to previous higher dose for 4 weeks then taper; consider DMARD

WHEN TO REFER TO A RHEUMATOLOGIST

Atypical features

- Age <60 years
- Chronic onset (>2 months)
- Lack of shoulder involvement
- Lack of inflammatory stiffness
- Red flag features: prominent systemic features, weight loss, night pain, neurological signs
- Peripheral arthritis or other features of muscle disease or connective tissue disease
- Normal or extremely high acute-phase response

Problems with steroids

- Incomplete or non-response to steroids
- Inability to reduce steroids or recurrent relapses
- Contraindications to corticosteroid therapy

References:

Dasgupta et al. BSR and BHPR guidelines for the management of polymyalgia rheumatic. *Rheumatology* 2010;49:186–190

Dejaco C, Singh YP, Perel P, et al. 2015 Recommendations for the management of polymyalgia rheumatica: a European League Against Rheumatism/American College of Rheumatology collaborative initiative. *Annals of the Rheumatic Diseases* 2015;74:1799-1807.

Patient information:

[Steroids | Side-effects, uses, time to work \(versusarthritis.org\)](#)

[Polymyalgia rheumatica \(PMR\) | Causes, symptoms, treatments \(versusarthritis.org\)](#)

See also:

[194FM Giant Cell Arteritis Guideline](#)

[Guideline 280FM Management of Patients on Immunosuppressants admitted with Suspected Infections](#)

[Polymyalgia rheumatica | Health topics A to Z | CKS | NICE](#)

Title of Guideline	Diagnosis and Management of Polymyalgia Rheumatica
Guideline Number	776
Version	3
Effective Date	October 2024
Review Date	October 2027
Original Version Produced	March 2012
<i>Approvals:</i>	
Rheumatology Consultants	22 nd July 2024
Medicines Check (Pharmacy)	15 th October 2024
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SDU(s)/Department(s) responsible for updating the guideline	Rheumatology
Uploaded to Intranet	22 nd October 2024
Buckinghamshire Healthcare NHS Trust	